

1. Administrative & Study Identification

Full Study Title: A Phase II, Randomized, Multicenter, Open-Label, Controlled Study of Tobemstomig Alone or in Combination With Tiragolumab Versus Atezolizumab in Patients With Previously Untreated Locally Advanced or Metastatic Urothelial Cancer Who Are Ineligible for Platinum-Containing Chemotherapy **Short Title/ Acronym:** A Study Evaluating Different Immunotherapies (LAG-3 and PD-1 With or Without TIGIT, Compared to PD-L1 Alone) in Participants With Untreated Locally Advanced Metastatic Urothelial Cancer **Protocol Number:** B044157 / 2023-504027-78-00 **NCT Number:** NCT05645692 **Posting ID:** 932585780473762236 **Sponsor/ Company Name:** Roche **Investigational Product:** Tobemstomig (R07247669) ± Tiragolumab vs. Atezolizumab (Trade Name: Tecentriq) **Phase of Development:** PHASE2 (Criteria Type: PHASE_REQUIREMENT - Trial must be in PHASE2) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor:** Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety of tobemstomig alone or in combination with tiragolumab compared with atezolizumab in participants with previously untreated, locally advanced or metastatic urothelial cancer (mUC) who are ineligible to receive a platinum-containing chemotherapy. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DoR), and characterize the efficacy profile of the treatment arms.

2.2 Background & Rationale To address the unmet medical need and improve clinical outcomes in patients with locally advanced or metastatic urothelial cancer who are ineligible ("unfit") to receive standard platinum-containing chemotherapy. This study investigates whether novel immunotherapeutic combinations (targeting LAG-3, PD-1, and TIGIT pathways) can provide superior safety, disease control, and response rates compared to standard-of-care PD-L1 inhibition (atezolizumab) alone.

3. Study Design & Methodology

3.1 Design Framework Study Type: INTERVENTIONAL **Control Type:** Active Comparator (Atezolizumab) **Blinding:** Open-label **Randomization:** Randomized

3.2 Planned Enrollment & Duration Targeted Enrollment: 204
evaluable patients **Number of Sites:** Multicenter (Global)
Estimated Study Start: 13-Apr-2023 **Estimated Primary Completion:**
31-Dec-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Eastern Cooperative Oncology Group (ECOG) Performance Status of ≤ 2 . 2. Histologically or cytologically documented locally advanced or metastatic transitional cell carcinoma (TCC) of the urothelium. Participants with squamous, sarcomatoid, micropapillary, and glandular variant histologies are eligible provided a urothelial component is present in the tumor specimen. 3. Ineligible ("unfit") to receive platinum-based chemotherapy. 4. No prior chemotherapy for inoperable locally advanced or metastatic or recurrent urothelial carcinoma (UC). 5. Measurable disease; at least one measurable lesion as defined by RECIST v1.1. 6. Availability of a representative leftover tumor specimen that is suitable for determination of PD-L1 status as assessed by a central laboratory. 7. Adequate hematologic and end organ function. 8. Negative for hepatitis B and hepatitis C virus (HCV). 9. Adequate cardiovascular function.

4.2 Exclusion Criteria 1. Pregnancy or breastfeeding. 2. $GFR < 15$ mL/min/1.73 m². 3. Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases or history of leptomeningeal disease. 4. Uncontrolled tumor-related pain. 5. Uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures. 6. Uncontrolled or symptomatic hypercalcemia. 7. Active or history of autoimmune disease or immune deficiency. 8. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on screening chest computed tomography (CT) scan. 9. Active tuberculosis (TB) or acute Epstein-Barr virus (EBV). 10. Significant cardiovascular/cerebrovascular disease within 3 months prior to initiation of study treatment. 11. Major surgical procedure, other than for diagnosis, within 4 weeks prior to initiation of study treatment, or anticipation of need for a major surgical procedure during the study. 12. History of another primary malignancy other than urothelial carcinoma within 2 years prior to initiation of study treatment (with the exception of malignancies with a negligible risk of metastasis or death). 13. Severe infection within 4 weeks prior to initiation of study treatment. 14. Treatment with therapeutic oral or intravenous antibiotics within 2 weeks prior to initiation of study treatment. 15. Prior allogeneic stem cell or solid organ transplantation. 16. Treatment with a live, attenuated vaccine within 4 weeks prior to initiation of study treatment, or anticipation of need for such a vaccine. 17. Current treatment with anti-viral therapy for HBV. 18. Treatment with any approved

anti-cancer therapy, including chemotherapy or hormonal therapy, within 3 weeks prior to initiation of study treatment. 19. Treatment with investigational therapy within 4 weeks or 5 drug-elimination half-lives (whichever is longer) prior to initiation. 20. Prior treatment with CD137 agonists or immune checkpoint blockade therapies, including anti-TIGIT and anti-LAG3 therapeutic antibodies or pathways targeting agents. 21. Treatment with systemic immunostimulatory agents within 4 weeks or 5 drug-elimination half-lives prior to initiation. 22. Treatment with systemic immunosuppressive medication within 2 weeks prior to initiation of study treatment. 23. History of severe allergic anaphylactic reactions to chimeric or humanized antibodies or fusion proteins.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Tobemstomig alone, Tobemstomig + Tiragolumab, or Atezolizumab alone administered in standard protocol-defined treatment cycles. **Drug Details**

(Control): Atezolizumab (Trade Name: Tecentriq, ATC Code: L01FF05). EMA Product Information: [EMA Link](#) **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until disease progression, unacceptable toxicity, withdrawal of consent, or loss of clinical benefit.

5.2 Concomitant & Prohibited Therapy Permitted: Prophylactic antibiotics (e.g., to prevent a UTI or COPD exacerbation) or oral antibiotics to treat an existing UTI. **Prohibited:** Systemic immunosuppressive medication, live-attenuated vaccines within 4 weeks of initiation, anti-viral therapy for HBV, and therapeutic oral/IV antibiotics within 2 weeks prior to study initiation.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging, PD-L1 Specimen Submission
Baseline	Day 0	Randomization, First Dose Administration, Baseline Vital Signs
Interim	Every 3-4 Weeks	IV Infusion, Safety Labs, Efficacy Assessment (RECIST v1.1 Tumor Imaging), AE tracking
End of Study	Follow-up	Final Vital Signs, Exit Interview, Disease Progression Assessment, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence and Severity of Adverse Events. **Measurement Method:** Continuous safety monitoring grading incidence and severity via clinical assessments, patient interviews, and safety labs.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR) based on investigator-assessed tumor response according to RECIST v1.1. 2. Progression-Free Survival (PFS). 3. Overall Survival (OS) and Duration of Response (DOR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection via patient interviews, safety labs, and clinical assessments prior to each treatment cycle. **Serious Adverse Events (SAEs):** Must be reported to the Sponsor/pharmacovigilance team within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent safety review committee to evaluate safety signals across the monotherapy and combination immunotherapy arms.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy analysis; Safety Set for all participants receiving at least one dose of the study drug to evaluate the primary outcome of AE incidence/severity. **Sample Size Justification:** Targeted Enrollment of 204 patients is powered to detect statistically significant differences in safety and efficacy between the experimental (tobemstomig ± tiragolumab) arms and the control arm (atezolizumab). **Handling of Missing Data:** Standard oncology imputation; right-censoring applied for time-to-event endpoints (PFS, OS) at the date of the last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits to ensure protocol adherence, RECIST v1.1 assessment compliance, and accurate safety reporting. **Audit Trail:** Strict eCRF locking and comprehensive data clarification processes managed by the Sponsor's clinical data management system.

11. Study Identifiers & Governance

Primary ID: B044157 **Registry Number:** NCT05645692 **Posting ID:** 932585780473762236 **Sponsor/Lead Organization:** Roche **Study Title:** A Study Evaluating Different Immunotherapies (LAG-3 and PD-1 With or Without TIGIT, Compared to PD-L1 Alone) in Participants With Untreated Locally Advanced Metastatic Urothelial Cancer **Official Regulatory Title:** A Phase II, Randomized, Multicenter, Open-Label, Controlled Study of Tobemstomig Alone or in Combination

With Tiragolumab Versus Atezolizumab in Patients With Previously Untreated Locally Advanced or Metastatic Urothelial Cancer Who Are Ineligible for Platinum-Containing Chemotherapy

12. Clinical Framework (Urothelial Cancer Specific)

Primary Condition: Urothelial Cancer **Preferred Name:** Suspected urothelial cancer / Urothelial Carcinoma **Semantic Type:** Finding
Disease Definition: Urothelial cancer is a type of malignancy originating from the urothelial cells that line the urinary tract. While most frequently found in the bladder, it can also develop in the renal pelvis, ureters, and urethra, representing the uncontrolled growth of these specialized epithelial cells.
Classification Codes: - **MeSH Code:** D001749 - **SNOMED ID:** 424564007 - **SNOMED Hierarchy:** 18873004 | Malignant neoplasm of urinary tract | Malignant neoplasm of urinary tract (disorder) **Medical Methodology:** Immunotherapy (Anti-PD-1/LAG-3 ± Anti-TIGIT vs Anti-PD-L1 [Atezolizumab - ATC: L01FF05]) **Optimization Target:** Safety (Incidence/Severity of AEs) and Disease Control

13. Study Procedures & Methodology

Management Pathway: Standard oncology pathway for Platinum-ineligible mUC **Education Protocol (HCP):** Site initiation visits and protocol administration training **Education Protocol (Patient):** Informed consent counseling and immunotherapy risk education **Quality Audit Mechanism:** Central pathology review for PD-L1 status and central RECIST v1.1 imaging verification

14. Observation & Follow-Up Schedule

Total Duration: Estimated ~44 months (April 2023 to December 2026) **Onsite Visit Cadence:** Every 3-4 weeks corresponding to infusion cycles **Remote Monitoring Frequency:** As needed per Sponsor SOPs **Checklist Review Interval:** Every 8-12 weeks for standardized tumor imaging assessments

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				